

Agent Handbook — 03 Clinical-Administration

Drafts clinical documentation to reduce clinician administrative burden — **draft only, no order entry**, with a clinician sign-off gate and 42 CFR Part 2 consent. Maturity: Demonstrated + Deployable-by-design; production-readiness (CSV/CSA, IdP, live connectors, pen test, HITRUST) is the engagement; AWS BAA precedes any PHI.

Workflow + human gate

Gather encounter context from the chart → **check 42 CFR Part 2 consent** before touching sensitive (SUD / behavioral-health) data; escalate if absent → **draft** the clinical note / summary (ehr.draft_note, draft only) → ground every cited fact → **suspend at the clinician sign-off gate**. **Named human gate**: Clinician sign-off. Ambient documentation has moved clinician burnout from ~52% to ~39% — but only with the clinician in control.

Systems / connectors (with standards)

- **EHR / FHIR** — Epic / Oracle Health via Amazon HealthLake (FHIR R4): encounter, observations, documents.
- **Consent ledger** — 42 CFR Part 2 / sensitive-scope consent check before disclosure.
- **Knowledge base** — clinical-content corpus for grounding.

Workforce roles to map in the IdP

CLINICAL_STAFF and PROVIDER (the sign-off gate). Map to custom:hpp_role.

Withheld authority (what it never does)

ehr.draft_note is draft-only — no order entry, no signed note. The agent never places an order, signs a note, or writes to the chart of record. It never discloses or acts on **42 CFR Part 2** data without an explicit consent grant.

Deploy (fixture mode, no PHI)

```
scripts/build_lambdas.sh
scripts/deploy.sh 03-clinical-administration dev portable native s3://my-cfn-
staging/hpp 10.32.0.0/16
```

- Params: infra/cloudformation/params/03-clinical-administration.json · Terraform: terraform apply -var-file=envs/03-clinical-administration.tfvars
- VPC CIDR 10.32.0.0/16.

Human-gate smoke test

1. Sign in as a seeded PROVIDER.
2. Drive a draft note on a record **without** Part 2 consent → confirm the agent **escalates** rather than disclosing.

3. With consent present, draft a note → run **suspends** at the clinician sign-off gate; HITL item appears.
4. Sign off → draft proceeds; approval written to the **append-only PHI-masked audit**.
5. Attempt order entry / a signed-note write → **denied by default** (draft-only).

Key regs

HIPAA Privacy/Security, HITECH; **42 CFR Part 2** (SUD/behavioral-health consent); 21st Century Cures (information blocking); clinical documentation-integrity policy.

Go-live checklist

- ☐ AWS BAA + SI BAA executed; no PHI before both.
- ☐ IdP federated; CLINICAL_STAFF / PROVIDER mapped to custom:hpp_role.
- ☐ Live EHR/FHIR connector validated (Epic/Oracle Health via HealthLake).
- ☐ **42 CFR Part 2 consent ledger** wired to the customer's consent source of truth.
- ☐ Guardrail + grounding tuned; CSV/CSA signed off; HITL SLA ratified.
- ☐ Pen test + HITRUST/SOC 2 evidence assembled; observability + runbooks adopted.